Talking with caregivers: Children react too

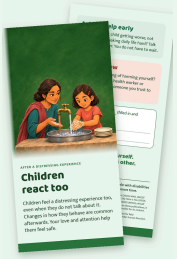
For people trained in psychological first aid, with a supervisor and known referral routes, including child protection.



Immediate danger or a child at risk? Pause the session, stay with the person and get urgent help now. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.



Poster A3



Leaflet, tri-fold

Your materials

Poster · a shared focus

For a group: open the discussion, invite observations and return to the main message. Make sure everyone can see; do not read every line aloud.

Leaflet · detail to take home

For a question or a closer look, and to keep or share. Open only the relevant section, for example the reactions by age group, and offer to read it together.

Use one or both, in any order. Focus on what caregivers need, not on finishing both materials.

Before people arrive

- ☐ Find out who the child protection focal point is in this area, and fill in and check the help contacts on page 2. Plan time after the session for private talks.
- ☐ Arrange safe play for children nearby, supervised by another adult, so that caregivers can join. Hang the poster where everyone can see it; bring enough leaflets.
- ☐ Plan language and access: simple words, large print, more time, or a sign language interpreter if needed and available. Invite caregivers of children with disabilities.

The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

Aim: caregivers know that changes in children are common, choose one way to help this week, and know where to get help.

1 Welcome and explain the purpose

2 min · either material

DO Introduce yourself and the topic. Joining is voluntary. Say clearly that nobody has to talk about their own child in the group.

SAY *"Today we will talk about how children react after a distressing experience, and the simple things families can do."*

2 Look at the picture together

3 min · poster if useful

DO Point to the adults and children in the picture. Explain that clinging, crying, anger or being very quiet are common. With the leaflet only, use its cover.

ASK *"What do you see the children doing?"*

SAY *"These changes are common. They do not mean the child is bad or that you are a bad parent."*

3 What helps

5 min · move between materials as needed

DO Go through the four things in the picture, from staying close to answering questions honestly. Ask caregivers to choose one to try this week.

SAY *"Children need adults' love and attention during difficult times."*

ASK *"What helps your children on a hard day?"*

4 When to ask for help

3 min · relevant help section

DO Point to the help messages. Say where the health worker and the child protection contact are; use page 2 if concern arises.

ASK *"If a child here needs help, who would you go to?"*

5 Check understanding and close

2 min · offer the leaflet to keep

DO Give out the leaflet and show *"What you may notice"* and *"When to ask for help"*. Mention the Nepali story book *My Hero is You* (ages 6 to 11). Offer to talk privately after the session.

ASK *"Which one thing will you try with your child this week?"*

Safety, referral and worker boundaries

Get urgent help now if someone:

SIGNS	WHAT TO DO
• Talks about suicide, or about harming themselves or someone else; this includes a child who talks about hurting themselves	1. Stay with the person. Do not leave them alone.
• Cannot keep themselves safe, or cannot care for a child in their care	2. Get urgent help now: a health worker, responder or the emergency service in this area.
• Seems very confused, or not aware of where they are	3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
• A child is alone or separated from their family	4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

A child may be at risk of abuse or neglect: follow your child safeguarding procedure and contact the child protection focal point. Do not investigate yourself.

If someone becomes upset

- Stay calm and stay close.
- Offer a quieter place away from the group, if they wish.
- Do not force them to talk. Ask what they need now.
- Stay as long as they want, or until someone they trust is with them.

PFA: Look, Listen and Link

Look: safety, urgent needs, people who are very distressed
Listen: ask about needs and concerns; help them feel calm
Link: basic needs, services, information, loved ones

Talk privately and link to further help if

- Changes in a child go on for a long time or get worse
- A child stops eating, playing or talking
- A caregiver says they cannot cope, or is afraid of hurting the child

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

Do

- ✓ Speak to caregivers with respect; they know their children best
- ✓ Give examples for different ages
- ✓ Include caregivers of children with disabilities
- ✓ Follow up privately with anyone who asks

Do not

- ✗ Blame parents for their children's reactions
- ✗ Ask about a child's experiences in front of the group
- ✗ Promise schooling, support or services you cannot confirm

Help contacts for this area

fill in and check before the talk

Health worker or facility _____

Urgent help _____

Child protection focal point _____

Other support _____



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO and UNICEF, Helping children cope with stress (2020); WHO, UNICEF and partners, Parenting for Lifelong Health tips (2020); IASC, Guidelines on MHPSS in Emergency Settings (2007); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015). This material draws on these publications; it was not created by WHO, UNICEF or IASC, which are not responsible for its content.

What this material is about

The poster and leaflet use everyday words, so that caregivers can talk about changes in their children without clinical labels. This page gives the meaning behind those words and the limits of your role; page 4 holds the clinical terms, for reference only.

Word key

ON THE POSTER OR LEAFLET	WHAT WE MEAN	HOW TO HANDLE IT IN THE TALK
"Children react too"*; *"Cling to you, sleep badly, cry or get angry more, become very quiet"	Common stress reactions in children, including regression (for example wetting the bed again) and changes in behaviour	Say *"changes"* or *"reactions"*, not *"problems"* or *"symptoms"*; do not blame parents
"Answer questions honestly, in simple words"	Age-appropriate, truthful information, including about a death or a missing relative	Help caregivers find simple words; do not advise hiding a death
"Do not punish a child for being afraid or upset"	Positive, non-violent parenting; fear is not misbehaviour	If a child may be at risk of harm, follow child safeguarding; do not investigate

Changes in children, and when to look further

Common changes

Clinging, crying more, sleeping badly, wetting the bed again, getting angry easily, being very quiet, or playing out what happened again and again. These are expected after a distressing experience.

Early help, at any time

If changes have not improved at all about one month after the event, are getting worse, or stop the child eating, playing or learning, link the family to a health worker. Sooner if the caregiver asks.

Get help now

If a child talks about hurting themselves, or a child may be at risk of harm, follow page 2 at once.

Keep these apart

- **Common reactions** are expected changes after a distressing experience, not an illness.
- **A protection concern** (abuse, neglect or separation) is not a mental-health label: it goes through child safeguarding.
- **A diagnosis** describes a lasting pattern that impairs daily life, assessed by a trained health worker.

Your limits

- You do not assess or diagnose a child.
- You do not investigate abuse or neglect; you report through the child safeguarding procedure.
- You do not promise schooling, support or services you cannot confirm.

Your role

This guide supports a session; it does not replace psychological first aid training. Use your training, supervisor and known referral routes, including child protection.

Look after yourself too: take breaks, talk with your supervisor or a colleague, and ask for support when you need it.



Talking privately with a caregiver

- Introduce yourself and ask permission. Explain what you can and cannot offer.
- Help them choose one thing to try this week, from the leaflet.
- Agree any follow-up only if it is possible and wanted; do not promise a visit or call you cannot make.
- Listen to their worries without judging; they know their child best.
- If they say they cannot cope or are afraid of hurting the child, take it seriously and link them to further help (page 2).

Sources: WHO and UNICEF, Helping children cope with stress (2020); WHO, Psychological First Aid: Guide for Field Workers (2011); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015); WHO, mhGAP module: Assessment and management of conditions specifically related to stress (2013); IASC, Guidelines on MHPSS in Emergency Settings (2007).

Clinical terms you may hear

For reference only. Do not use these terms with caregivers, and do not diagnose. They help you understand what a health worker may say, and why early help does not have to wait.

TERM OR SITUATION	WHAT THE FACILITATOR NEEDS TO KNOW	SOURCE
Common reactions in children	Clinging, crying, sleep problems, wetting the bed again, irritability, withdrawal, or playing out the event. Expected after a distressing experience.	WHO and UNICEF, Helping children cope with stress (2020)
When to look further	Changes that have not improved at all about one month after the event, are getting worse, or stop the child eating, playing or learning: link to a health worker. A child who talks about hurting themselves: urgent help.	WHO mhGAP stress module (2013)
Post-traumatic stress in children	Can occur in children; the criteria for children aged six and younger differ from those for older children and adults. Assessed only by a trained health worker.	American Psychiatric Association, DSM-5-TR (2022)
Child protection	Any concern of abuse, neglect or separation is a protection matter, not a mental-health label: follow child safeguarding and contact the child protection focal point.	IASC, Guidelines on MHPSS in Emergency Settings (2007)
***"Trauma"*, when a caregiver uses the word**	Do not correct the word and do not start a discussion about trauma. Ask what the caregiver means and what the child does: *"When you say traumatised, what do you notice in your child?"* Caregivers often use the word for a specific change, for example a child who will not let them out of sight, or who is afraid of loud noises at night. Such changes are common reactions after a frightening or potentially traumatic experience. Respond to what the caregiver describes, and use the early-help route if it continues, gets worse or makes daily life hard.	Facilitation practice for this series; listening as in WHO, Psychological First Aid (2011)

Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

Words for you to say or ask · What the person may say · Guidance for you (plain text)

WHEN A CAREGIVER SAYS "TRAUMA"

> *"My daughter is traumatised. She will not let me out of her sight."*

Poster: children may cling to you.

1 Ask what they mean

ASK — *"When you say traumatised, what do you notice in her?"*

THEY SAY — *"She cries when I go to the queue for food. At night she holds my hand until she sleeps."*

2 Name it as common

SAY — *"Many children cling to their parents after a frightening time. It shows she feels safest with you."*

3 Agree one thing to try

DO — Suggest telling her where you are going and when you will be back, and keeping regular times for meals and sleep, as on the leaflet.

4 Know when to look further

DO — If there is no improvement at all about one month after the event, or it gets worse, or she stops eating, playing or talking, link to a health worker (page 3). Sooner if the caregiver asks.

WHEN A CHILD WETS THE BED AGAIN

> *"He is eight and he wets the bed again. I shouted at him this morning."*

Poster: do not punish a child for being afraid or upset.

1 Listen without blaming

DO — Thank the caregiver for telling you. Do not blame them or the child.

2 Explain

SAY — *"Wetting the bed again can happen after a distressing experience. It is not naughtiness, and it is not your fault."*

3 Agree a calm response

DO — No punishment or teasing; give extra time and affection, as on the leaflet. If the child has pain or fever, or seems unwell, link to a health worker.

WHEN A CAREGIVER CANNOT COPE

> *"I have no patience left. Sometimes I am so angry I want to hit him."*

Leaflet: look after yourself too.

1 Listen, then ask about safety

DO — Thank them for saying it, without judging.

ASK — *"Has anyone been hurt? Are you afraid you might hurt him?"*

DO — If a child may be at risk of harm, follow your child safeguarding procedure (page 2). Do not investigate yourself.

2 Offer a pause

SAY — *"When anger comes, take a short pause: breathe, then respond."*

3 Share the load and link

DO — Help them think who could share care, such as family or neighbours. A caregiver who says they cannot cope is a reason to link to further help (page 2).